PayMedix

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To whom it may concern:

Thank you for the opportunity to provide input to the Consumer Financial Protection Bureau (CFPB), U.S. Department of Health and Human Services, and U.S. Department of Treasury (the "Agencies") regarding medical payment products (Request for Information published in the Federal Register dated July 12, 2023).

The Agencies should be lauded for their examination of the way in which medical payment products are being utilized beyond their original purpose for assisting with costs generated from out-of-network and elective care. (The RFI defines "medical payment products" as those "being administered by financial services companies, who manage the billing and collections process for these products *and earn revenue through interest and fees".)*

The expanded use of medical payment products imposes an even greater financial burden on many patients, limits their creditworthiness, and raises overall healthcare costs. The Agencies should ensure full transparency for all medical credit cards, installment loans, and other medical payment products that carry interest charges and consumer fees and can cause significant increases in cost of care. Further, the Agencies should ensure that consumers have clear

information regarding charity care eligibility and preserve their ability to negotiate bills and secure adjustments.

We have a crisis of healthcare affordability in the U.S., which is a bigger topic and which a myriad of public policy and private market solutions have attempted to address. In the meantime, it is necessary for patients to pay for their rapidly rising portion of healthcare costs, especially in-network allowed charges such as deductibles and co-pays. The good news is that new products have entered the market and created a stronger and more equitable outcome for consumers. Innovative private companies such as PayMedix offer consumer-friendly alternatives which simplify the medical billing experience, treat all patients equitably and with dignity, and eliminate additional costs for patients and family members.

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1. Background

U.S. healthcare expenditures, as measured by the annual National Health Expenditure data published by CMS, totaled $4.3 trillion in 2021. This represents 18% of U.S. GDP, up from 13% in

2000, 9% in 1980 and 5% in 1960.[1] While the portion of total U.S. healthcare costs not covered by insurance and therefore paid "out of pocket" by patients declined on average from 47% in 1960 to 10% in 2021, the total dollars owed by patients has grown 5.9% annually over the same period, which is nearly twice the rate of U.S economic growth. In 2021, the NHE data shows out of pocket spending grew 10.4% to $433.2 billion.

The crisis of healthcare affordability is well documented, and all stakeholders have had to adapt. Employers have increasingly shifted rising costs to employees through High Deductible Health Plans (HDHPs), higher co-pays and higher monthly premiums. Healthcare providers have tightened up claims processing over the years, and as the amounts due from consumers continues to rise, they have introduced easier ways for patients to pay (online portals, card on file, text-to-pay) and extended payment terms. Unfortunately, the financing options implemented by providers at times have hurt consumers' ability to negotiate bills and amounts due, reduced their access to financial assistance/ charity care, and brought in unfavorable financing terms (interest charges and fees) that are not transparent to patients. This has further reduced access to care.

In 2022, four in ten adults (43%) said that they or a household member put off or postponed needed health care due to cost.[2] When they do receive care, patients are stressed by paying the bill. In a recent study, over one-half of Americans said that in the last six months paying for medical bills has been stressful (52%) and that stress has had some impact on their physical or mental health (92%).[3]

Another recent report indicated how widespread the problem has become, with 23.5% of middle-class people (nearly 17 million) having medical debt in 2020, which was more than the 22% of lower-income population.[4] Making matters worse, healthcare providers may offer so-called "in-house payment plans" to patients to pay off their hospital charges over time, and then turn around and deny care to patients who have unpaid balances outstanding.[5]

It is also important to make sure consumers understand their medical bills, and can dispute hospital, physician, and other charges when they believe they are incorrect. In one recent study, 59% of providers rated patient financial literacy as "poor" despite their own investments in patient educational materials and financial counselors. [6] A separate study found that 56% of

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[1] NHE website and "NHE Fact Sheet" (2021 data), *available at* https://www.cms.gov/research-statistics-data-and-systems/statistics-trends-and-reports/nationalhealthexpenddata/nhe-fact-sheet.
[2] Peterson-KFF Health System Tracker, "How does cost affect access to healthcare?" (Jan. 2023), *available at* https://www.healthsystemtracker.org/chart-collection/cost-affect-access-care/.
[3] PayMedix, "Healthcare Payments and Financial Disparities Study" (Jun. 2023), *available at* https://paymedix.com/wp-content/uploads/2023/06/PayMedix-National-Survey-Key-Finding-Final.pdf.
[4] Third Way, "Medical Debt Hits the Heart of the Middle Class" (Aug. 2023), *available at* https://www.thirdway.org/report/medical-debt-hits-the-heart-of-the-middle-class.
[5] New York Times, "This Nonprofit Health System Cuts Off Patients With Medical Debt" (Jun. 2023), *available at* https://www.nytimes.com/2023/06/01/business/allina-health-hospital-debt.html.
[6] "Study Finds Healthcare Providers Looking to Outsource Revenue Cycle Management Processes" (Apr. 2023), *available at* https://www.carecredit.com/pressrelease/cwh-advisors-patientpay-research/.

Americans feel "completely lost" when it comes to understanding their health plans.[7] In one consumer survey, 38% of patients said their medical bills were somewhat confusing or extremely confusing.[8] Any solutions to help patients afford healthcare should include help to improve patients' abilities to understand what they owe and advocate for themselves on the financial side of healthcare.

This is especially important because medical billing errors occur frequently – half of respondents to a recent study reported incorrect charges of at least $200, and a quarter saw a difference of more than $500 on their bills.[9] When patients use medical payment products, including credit cards and installment loans that convert medical debt to consumer debt, they often lose the ability to go back and ask for adjustments to incorrectly billed amounts. In fact, once a billed amount moves over to a medical payment product, some interpretations hold that this represents a consumer's commitment that the charge was justified.[10] Effectively once an incorrect billed amount is paid on a medical credit card or installment product, some of those payment products require the patient to ultimately pay that incorrect amount, even if the patient was erroneously overbilled.

There are programs available to help lower-income patients pay their medical bills, including each non-profit hospital's own financial assistance policies and enrollment in state Medicaid plans. However, healthcare providers often do a poor job making patients aware of these options, and they continue to send bills to patients who could potentially qualify for assistance. One study demonstrated that almost half of non-profit hospitals sent medical bills to patients whose incomes are low enough to qualify for charity care, accounting for $2.7 billion in medical expenses which would have been written off according to the hospital's own policies if the patients had applied for assistance.[11]

Historically, consumer healthcare costs in the commercial market (i.e., those not in a government health care program) have been dealt with in two ways:

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[7] "More than Half of Americans Confused by Health Insurance, Including HSAs" (Feb. 2021), *available at* https://www.bendhsa.com/newsroom/more-than-half-of-americans-confused-by-health-insurance-including-hsas.
[8] "Nearly 40% of Americans Confused by Medical Bills" (Dec. 2022), *available at* https://akasa.com/press/nearly-40-of-americans-confused-by-medical-bills/.
[9] "New Research Reveals Consumer Frustrations with Healthcare Billing and Lack of Processes for Correcting Errors" (Jan. 2023), *available at* https://www.zelis.com/news/new-research-reveals-consumer-frustrations-with-healthcare-billing/.
[10] NPR, "Doctors And Hospitals Tell Patients: Show Us The Money Before Treatment" (Dec. 2016), *available at* https://www.npr.org/sections/health-shots/2016/12/07/504589131/doctors-and-hospitals-tell-patients-show-us-the-money-before-treatment.
[11] KFF Health News, "Patients Eligible For Charity Care Instead Get Big Bills" (Oct. 2019), *available at* https://kffhealthnews.org/news/patients-eligible-for-charity-care-instead-get-big-bills/.

1. **In-network allowed charges**. These are the copays, deductibles, and out-of-pocket amounts for which consumers are responsible under their employer-provided health coverage and which received the best-negotiated price with healthcare providers
    - A combination of the provider, insurer, and employer could offer financial assistance to consumers to pay their portion (*e.g.,* payment plans, HRAs/ FSAs/ HSAs, health/ wellness incentive accounts, etc.)
    - Assistance was both **guaranteed and universal** for all consumers regardless their individual financial status
    - Consumers have a financial incentive to seek out cost-effective care

2. **Out-of-network and elective care.** This is care that consumers choose to consume even though it will cost them more money and/or is not a covered medical benefit (e.g., cosmetic surgery)
    - If financed, the consumer could individually seek a credit product, or the healthcare provider could partner with a financial services firm to offer that function
    - Consumers are **individually underwritten**, and whether credit is offered and if so on what terms varies significantly
    - Consumer interest and fees are typically a key component of these products

Medical payment products as defined in the RFI were traditionally sold for use in the out-of-network and elective care, and they filled a role in that arena. More recently, though, these medical payment products are being utilized to finance in-network allowed costs after insurance.

Many, including PayMedix, view this expansion of medical payment products as problematic. **Any product for in-network allowed charges that does not apply to all individuals (regardless of their creditworthiness) and provide full coverage up to a patient's out-of-pocket maximum is cherry-picking consumers with higher credit ratings. This approach diminishes universal access to care, increases health inequity, and drives up overall healthcare costs.** A better approach for assisting consumers with their in-network allowed charges is a "Guaranteed Universal Billing and Payment Service." We define and explain this term in Section 2.

2. The PayMedix Guaranteed Universal Billing and Payment Service

An alternative model exists that does not place additional financial burden on patients, preserves their creditworthiness, and treats all patients equitably and with dignity. This Guaranteed Universal Billing and Payment Service is being utilized by a growing number of

patients and is addressing the payment challenges faced by patients covered by health plans – since 58% of provider bad debt is from patients with health insurance, a 5x increase in 3 years.[12]

In this model, employers contract with a third party to reimburse healthcare providers for the out-of-pocket portion of the in-network medical bill owed by their employees. The third-party, for example PayMedix, is then responsible for collecting the out-of-pocket amount owed from the patient, who receives the PayMedix SuperEOB combining all billing and EOB information from healthcare providers they visit in one consolidated monthly statement.[13] Patients see immediately what they owe, how to pay it, and how to get credit if they need it in the form of extended payment options. The provider is not involved in collecting balances due from these patients.

Guaranteed. Payment is guaranteed to the providers on behalf of all consumers.

Universal. All employees and their dependents are automatically included, and all in-network costs up to the full out-of-pocket maximum can be included. No one is excluded for any reason.

Billing. All of the consumer's medical bills and EOBs are reconciled automatically and are replaced by a simple monthly summary statement.

Payment. Consumers can pay one bill that covers all of the participating providers, and if they need to pay over time then they can automatically get a no-fee, 0% interest payment plan.

Service. Since PayMedix is funded by employers and providers, it is provided at no cost to the consumer.

a) Reducing Employee Financial Stress

Employers can play a large role in helping employees reduce the stress of healthcare billing. 60% of employees say their employers are responsible for providing financial strategies to help them manage expenses, while 72% of corporate benefits managers are interested in a simplified medical payments solution, especially payment options that are flexible and offer 0% or low interest. [14] Since the PayMedix Guaranteed Universal Billing and Payment Service guarantees 0% interest credit for all patients and has no consumer fees, it is an excellent fit for both benefits managers and employees.

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[12] "Hospital collection rates for self-pay patient accounts" (Aug. 2022), *available at* https://www.crowe.com/-/media/crowe/llp/widen-media-files-folder/h/hospital-collection-rates-for-self-pay-patient-accounts-report-chc2305-001a.pdf.

[13] Employees are able to use the PayMedix simplified billing service for medical claims generated with healthcare providers who are in-network for their health plan, and also in the PayMedix network.

[14] PayMedix, "Healthcare Payments and Financial Disparities Study" (Jun. 2023), *available at* https://paymedix.com/wp-content/uploads/2023/06/PayMedix-National-Survey-Key-Finding-Final.pdf.

Employers offer the PayMedix Guaranteed Universal Billing and Payment Service to their employees to supplement their commercial group health benefit plan, and provide a benefit to employees who have moved to a High Deductible Health Plans (HDHPs). Patients with a HDHP incur higher out of pocket medical expenses until they meet their annual deductible. Unlike medical payment products that are offered to some patients after they receive care, all patients enrolled in PayMedix know before they receive care that they have the flexibility and benefits of the PayMedix service. This certainty removes a stubborn and persistent source of a barrier to care and allows the patient to proceed with needed medical services.

The characteristics and demographics of participating employees in the PayMedix billing service are representative of the full cross section of the employer's workforce. The billing solution is the same for the CEO of the firm as it is for an entry level hire. The PayMedix billing service is uniform across the participating workforce of each client. This includes core service features such as the configuration of the consolidated bill ("SuperEOB"), and the flexibility of the employee to choose the most appropriate bill repayment plan based on their individual financial situation.

Since participants know they will be provided care without providers checking to see if they can afford it or have outstanding medical debt, PayMedix improves access to care. The United States has seen a recent spike in later-stage disease because consumers were unable or unwilling to receive regular care during COVID; people now show up with Stage 4 cancer instead of getting caught earlier. This dynamic has always been the case for those with concerns about how they would be able to pay their portion of in-network allowed charges. PayMedix removes that concern – its employer clients have seen annual medical trend in the 2.5-3.4% range for years, or about half the market norm. When people know they can access care, they get more care when they need it. This saves the healthcare system overall as well as improves the health status of the individual patients.

b) Health Equity

It is estimated the 43% of Americans avoid seeking care when they need it[15] cut back on medications to treat chronic conditions like diabetes or fail to follow through on important imaging and other tests that can save lives. They do this because they don't have the means to pay for care.

An important feature of the PayMedix model is that unlike medical credit cards and installment loans targeted for post-service medical expenses, PayMedix does not require each patient to apply or be approved to qualify for the service. There is no review of employee credit history during enrollment, and no looking at other data sources that would be used to risk score patients and then adjust pricing and other terms at the individual level. Thus, PayMedix does not 'cherry pick' low-risk patients or seduce high-risk patients with gimmicky teaser rates. When an employer selects PayMedix as a benefit, all its employees are offered credit up to their

[15] *Ibid.*

annual in-network out of pocket maximum with 0% interest and no fees to the patient. The cost of the service is borne by the providers and employers.

While a medical credit card typically offers its revolving line of credit to very few subprime consumers (<620 credit score), 25% of PayMedix participants are in the subprime category. This includes 10% with a credit score less than 480. Most of these individuals would not access needed care without a Guaranteed Universal Billing and Payment Service like PayMedix, which would suboptimize health outcomes and drive up overall healthcare costs since patients will end up sicker and ultimately utilize more expensive care.

Medical payment products often find patients at a moment of great weakness – *e.g.*, when they find out that they need surgery - without the ability to evaluate options or make a thoughtful financial decision. With the PayMedix program already in place, participants need never worry about provider-facilitated medical payment product being offered to them when they can least afford to make a bad choice. There is no need since PayMedix will have automatically already handled the transaction. This is good for the patient, good for the healthcare provider, and good for society at large.

c) Patient Benefits I: Simplified Billing Process

Financial literacy with healthcare billing and insurance remains a chronic and systemic problem in the United States, with an estimated two-thirds of Americans unable to correctly define healthcare concepts and products like insurance premiums, co-payments, deductibles.[16] This confusion and lack of knowledge contributes to consumers delaying payment to providers, as they are often uncertain if the balance due is fully adjudicated and final. In the PayMedix solution, consumers are presented with a simplified, yet comprehensive consolidated bill (SuperEOB). This statement clearly summarizes the balance after insurance from all adjudicated medical claims, as well as the total amount of payments that have already been made to in-network providers on the employee's behalf. This single monthly statement combines the detail of an Explanation of Benefits (EOB) with the attributes of a conventional "monthly billing statement."

This simplification is critical. Today, the healthcare system overall spends a fortune to inundate consumers with complicated information that does not reconcile or make sense to most consumers. Those who can and will pay their medical bills do so, but only eventually. Experienced consumers know that most of what they receive will not make sense and that they should wait until it is all sorted out. In other words, we as a healthcare system have trained consumers to ignore us.

Healthcare cost and quality improvement require consumers to be engaged. Each provider implementing yet another billing or payment point solution could incrementally increase each

[16] Forbes, "U.S. Consumers Often Lack Literacy In Healthcare Finances" (Jun. 2021), *available at* https://www.forbes.com/sites/joshuacohen/2021/06/01/us-consumers-often-lack-literacy-in-healthcare-finances/.

of their financial results for a time, but the overall increase in the volume of consumer billing and payment outreach is driving consumers away. Think of someone getting surgery – the surgeon, the anesthesiologist, radiologist, pathologist, and facility will all bill and ping the consumer multiple times with a confusing array of often conflicting information. Without a simple, single Source of Truth like the PayMedix SuperEOB, consumers are left confused, and providers are left to spend a fortune on increasingly intrusive and expensive collection efforts that still do not generate great yield.

The PayMedix Guaranteed Universal Billing and Payment Service includes making the patient's out of pocket payment to healthcare providers on behalf of the employee. The consumer then pays PayMedix for the out-of-pocket amount that has been paid on their behalf. The consumer can reimburse PayMedix in full upfront or can create a payment plan to repay PayMedix over time. Unlike medical credit cards or installment loan products, repayment plans include no interest charges, late fees, hidden costs tied to defined promotional offers or retro-effective interest schemes if payment dates are missed. The employee is simply reimbursing for the face value of the medical bills already paid on their behalf by PayMedix. There is no cost to the employee to participate in the PayMedix solution. This is offered as an employer-subsidized program to all employees regardless their salaries, demographics, or credit histories.

d) Patient Benefits II: Reduced Cost and Risk

The PayMedix model does not charge any interest or fees to patients. There are no hidden fees, no teaser rates, and no deferred accrued interest. The cost of the PayMedix billing simplification service is covered by employers and healthcare providers. Employers pay a fee to offer this service to their employees. Providers receive a discounted amount of the post-adjudication patient balance and receive guaranteed payment from PayMedix. Patients pay nothing extra, and nothing in the PayMedix model contributes to increases in healthcare costs or medical inflation.

If a patient needs a payment plan, PayMedix develops a payment plan that works for the individual's particular needs and covers all the costs. This never includes interest charges, late fees, origination fees or any other additional expenses for the patient.

PayMedix also supports all employer programs to encourage employees to seek less expensive care. This may include, for example, bundled payment programs, zero or low co-pay services at more efficient providers, and incentive programs (e.g., healthy living/ wellness or going to efficient providers).

e) Provider Benefits I: No Promotional Role

Medical credit cards and installment loans targeted for healthcare expenses are complicated financial products. Historically, patients have tended to place a high level of trust in their clinicians and their staff. As a result, companies offering medical payment products have often

relied on the provider to introduce and even recommend their products to patients. Promoting or at least attempts to guide their patients on selecting the most appropriate medical billing oversteps the standard administrative and collections responsibilities of a provider organization. It introduces the risk that a patient may receive unwanted or unneeded care that they cannot afford.

PayMedix and similar models are not medical payment products and do not involve the provider's staff in driving enrollment or communicating the benefits of participation to patients. PayMedix is a simplified billing service offered by employers to all employees, with no incentives, inducements, or other features to motivate providers to push patients into unnecessary care or extra financing costs.

f) Provider Benefits II: Reduced Administrative Burden

Administrative expenses in the U.S. healthcare system take many forms, but one of the most prominent is the process of validating and tabulating amounts owed from providing patient care, and then sending bills and collecting amounts due from patients. Hospitals, physician offices, surgery centers, labs, imaging centers, and other clinical facilities have invested in people and technology tools to accomplish these tasks.

While the adoption of new revenue cycle technology and service solutions by clinical facilities has eased some of the burden of processing medical insurance claims, collecting balances owed by consumers remains a universal and expensive challenge for healthcare providers. Billing teams have invested in portal technologies to enable patient self-service, added payment innovations such as "card on file" to securely store credit card information for future payments, and started to utilize text messaging and robo-dialing to reach patients with overdue balances. Medical billing staff spend an enormous amount of time and money chasing patients with outstanding balances, and risk negatively impacting their patient relationships with the use of intrusive communication tools.

The PayMedix model reduces the administrative strain and costs of patient collections for an individual provider and avoids damaging the provider-patient relationship. In exchange for a modest fee to participate in the PayMedix simplified billing solution, providers avoid incurring the costs associated with collecting overdue balance from patients participating in the PayMedix model. Providers send fewer billing statements and make fewer phone calls and are able to use their resources to focus on clinical care and ensure the best possible patient outcomes. Providers also appreciate that the PayMedix model is a non-recourse arrangement, so they have certainty of payment with no risk that non-performing patient accounts will "boomerang" back to their balance sheet.

Furthermore, the PayMedix model helps to reduce the cost of patient collections across <u>all</u> participating providers. PayMedix centralizes patient collection activities in one organization and operates a compassionate, efficient collection process at scale. This lowers the unit cost of

overall patient collections below what each provider would achieve doing these activities on their own.

g) Charity Care

Unlike medical credit cards and installment loans targeted for medical expenses, providers assume no role in enrolling patients in the PayMedix service. Participating patients are enrolled in PayMedix through their employers. The providers play no role in introducing or incenting patients to participate in the PayMedix service.

This means providers have no conflicted interests when identifying and helping their patients access other local, regional, or federal financial assistance programs. Importantly, the financial assistance process is not hindered by the patient's participation in the PayMedix billing solution service. Furthermore, PayMedix helps consumers gather the necessary information and apply for financial assistance from healthcare providers. As of July 2023, PayMedix has assisted patients in qualifying for $750,000 of charity care for over 5,000 individual medical claims. This advocacy offered by PayMedix is communicated on the firm's SuperEOB and elsewhere, and in these cases -- and with the consumer's permission -- PayMedix can assemble documents and aggregate key qualifying information to the health system on behalf of and with no fee to the consumer. PayMedix is not aware of examples in which medical credit cards and installment loan vendors intervene for their customers to identify charity care relief.

h) Disputed Medical Bills

With the PayMedix billing simplification service, patients can always challenge a bill. The PayMedix customer service team will support the patient in understanding and ultimately disputing any bill with which they disagree. Most customer disputes center on whether the claims should have been paid in the first place. Determining the correct answer to this question is the responsibility of the health plan. PayMedix will work with the health plan and the consumer if the consumer believes a claim should not have been paid and their out-of-pocket amount should have been $0. If the consumer is correct, then PayMedix will process any adjustments and refund the appropriate amount of money to the consumer.

i) Consumer Protections

While the three major U.S. credit reporting companies recently agreed to remove medical debt of less than $500 from credit reports, this only begins to address the way these balances affect patient credit. According to the U.S. Census, in 2017, 19% of U.S. households carried medical debt and the median amount owed was $2,000.[17] One source estimates that most Americans

[17] U.S. Census, "19% of U.S. Households Could Not Afford to Pay for Medical Care Right Away" (Apr. 2021), *available at* https://www.census.gov/library/stories/2021/04/who-had-medical-debt-in-united-states.html.

with medical debt owe over $1,000.[18] A substantial number of Americans will still see medical debt negatively affect both their credit scores and their ability to access financing for any needs. It is necessary to find solutions to address the high cost of health care without pushing more consumers to take on onerous and high-interest debt.

Unpaid balances are also considered medical debt in the PayMedix model, not consumer debt as is the case with many types of medical payment products.

3. Response to RFI Questions

To better understand the medical payment product market, the agencies seek data and comments on the interest and fee costs of these products (including both interest rates and total accrued interest), the application and approval process for these products, and trends of medical payment product use.

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model. "Medical payment products" is defined in the RFI as those "being administered by financial services companies, who manage the billing and collections process for these products and earn revenue through interest and fees." We do not charge patients any interest or fees. We would suggest adding the word "limited" before "products" in this definition since many medical payment products are limited to certain individuals (e.g., those with higher credit scores), limited in duration (e.g., 12 months), and/or limited by a dollar cap (e.g., $2,000). The PayMedix model has none of those limitations.

The PayMedix Guaranteed Universal Billing and Payment Service does not charge any interest or fees to patients. There are no hidden fees, no teaser rates, and no deferred accrued interest. The cost of the PayMedix service is covered by employers and healthcare providers. Employers pay a fee to offer this service to all their employees. Providers receive a discounted amount of the post-adjudication patient balance and receive guaranteed payment from PayMedix. Patients pay nothing extra, and nothing in the PayMedix service contributes to increases in healthcare costs or medical inflation. If a patient needs a payment plan, PayMedix develops a payment plan that works for the individual's particular needs and covers all of the costs. This never includes interest charges, late fees, origination fees or any other additional expenses for the patient.

The PayMedix Guaranteed Universal Billing and Payment Service does not exclude any consumer at employer clients for any reason, including credit score. All in-network allowed charges are included, all the way to the out-of-pocket maximum. Payment plans are customized for each individual, with PayMedix average currently 17 months.

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[18] KFF-Peterson Health System Tracker, "The state of the U.S. health system in 2022 and the outlook for 2023" (Dec. 2022), *available at* https://www.healthsystemtracker.org/brief/the-state-of-the-u-s-health-system-in-2022-and-the-outlook-for-2023/.

The agencies also seek information as to the total outstanding consumer debt on medical credit cards, medical installment loans, and other medical payment products.

No response provided.

Data regarding the characteristics and demographics of medical payment products users is also welcome, such as whether users are insured or uninsured, whether certain populations or income groups are more likely to use these products, and whether use is concentrated in certain geographies or for patients seeking particular kinds of care.

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

The PayMedix Guaranteed Universal Billing and Payment Service is offered to consumers with health insurance. These consumers receive their health insurance through an employer's group health insurance plan. An important feature of the PayMedix service is that unlike medical credit cards and installment loans targeted for post-service medical expenses, PayMedix does not require each patient to apply or be approved to qualify for the service. There is no review of employee credit history during enrollment, and no looking at other data sources that would be used to risk score patients and then adjust pricing and other terms at the individual level. Thus, PayMedix does not 'cherry pick' low-risk patients or seduce high-risk patients with gimmicky teaser rates. When an employer selects PayMedix as a benefit, all of its employees are offered credit up to their annual out of pocket maximum with 0% interest and no fees to the patient. While a medical credit card typically offers its revolving line of credit to very few subprime consumers (<620 credit score), 25% of PayMedix participants are in the subprime category. This includes 10% with a credit score less than 480.

The agencies also seek to better understand the level of concentration in the medical payment product market, the ownership of medical payment product companies (including ownership by health care providers, health insurance issuers, or private equity firms), and the implications of these factors for competition and consumer choice.

No response provided.

To that end, the agencies seek specific information on the types of financial entities that offer medical credit cards and loans.

No response provided.

The agencies seek to understand to what extent medical credit cards and loans may hamper financial assistance and access to benefits, and any options for regulators to reduce such barriers.

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

Unlike medical credit cards and installment loans targeted for medical expenses, providers assume no role in enrolling patients in a Guaranteed Universal Billing and Payment Service like PayMedix. Participating patients are enrolled in PayMedix through their employers. The providers play no role in introducing or incenting patients to participate in the PayMedix service.

This means providers have no conflicted interests when identifying and helping their patients access other local, regional, or federal financial assistance programs. Importantly, the financial assistance process is not hindered by the patient's participation in the PayMedix service. Furthermore, PayMedix helps participating consumers gather the necessary information and apply for financial assistance from healthcare providers. This is provided at no cost to the consumers. As of July 2023, PayMedix has assisted patients in qualifying for $750,000 of charity care for over 5,000 individual medical claims. This advocacy offered by PayMedix is communicated on the firm's SuperEOB and elsewhere, and in these cases -- and with the consumer's permission -- PayMedix can assemble documents and aggregate key qualifying information to the health system on behalf of the consumer. PayMedix is not aware of examples in which medical credit cards and installment loan vendors intervene for their customers to identify charity care relief.

The agencies also seek to understand the extent to which health care providers, including tax-exempt hospitals, screen patients for public or private insurance eligibility, financial assistance eligibility, or other benefits before offering them medical credit cards or loans.

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model. PayMedix does offer a Guaranteed Universal Billing and Payment Service. In this model, all providers simply confirm private insurance eligibility. PayMedix is available to all members of an employer's health plan, and the providers do not 'offer' the consumers anything. The consumers were already enrolled via their employer before ever getting care.

It is unclear to PayMedix how providers could determine financial assistance eligibility before providing care. So much information is typically required – some that will not even be known by the provider at the point of care – that care delivery would be slowed, and consumers' health would be put at further risk if providers were to be required to determine financial assistance eligibility before providing care.

In the PayMedix model, there are no delays in delivering care and providers do not need to attempt to determine financial assistance eligibility up front. Providers simply confirm consumers' PayMedix eligibility and then know that that consumer balance will either be paid or that PayMedix will have, with the consumer's permission, helped the consumer get financial assistance from the provider. This service is provided at no cost to consumers.

The agencies additionally seek comment on how frequently patients discover billing errors after signing up for a medical payment product, the main sources of billing errors, and how paying medical bills via a medical payment product affects patients' ability to dispute those bills. The agencies seek comment on how to ensure that patients retain their rights to challenge inaccurate bills regardless of payment method.

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

PayMedix does offer a Guaranteed Universal Billing and Payment Service. With the PayMedix model, patients can always challenge a bill. The PayMedix customer service team will support the patient in understanding and ultimately disputing any bill with which they disagree. Most customer disputes center on whether the claims should have been paid in the first place. Determining the correct answer to this question is the responsibility of the health plan. PayMedix works with the health plan and the consumer if the consumer believes a claim should not have been paid and their out-of-pocket amount should have been $0. If the consumer is correct, then PayMedix will process any adjustments and refund the appropriate amount of money to the consumer.

It should be noted that consumer confusion can also play a role here. Typically, consumers are inundated with EOBs ("Explanations of Benefits") from their insurance plan, medical bills and "this is not a bill" statements from each provider they used. Much of the information appears contradictory or incomplete to consumers, and so they wait to see how it all sorts out over time.

In the PayMedix model, the consolidated monthly statement incorporates the information that populates standard EOBs as well as that on provider bills. It is all in one place, and reconciled for the consumer. The consumer can certainly challenge any charge, as explained elsewhere in these comments. It is clear, however, exactly what the charge truly was.

The agencies also seek comment on incentives offered by financial companies to health care providers for their promotion of medical payment products, including revenue-sharing and other incentives.

The PayMedix Guaranteed Universal Billing and Payment Service and similar models do not involve the provider's staff in driving enrollment or communicating the benefits of participation to patients. The PayMedix service is offered by employers to all employees, with no incentives, inducements, or other features to motivate providers to push patients into unnecessary care or extra financing costs.

The agencies are also interested in any training or other support that medical payment product companies offer to providers. The agencies are interested in whether such incentives or support might implicate the Federal anti-kickback statute or other laws or regulations.

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

Provider staff is trained to understand the different billing process for patients who participate in the PayMedix Guaranteed Universal Billing and Payment Service, and those who do not utilize PayMedix. When patients utilize the PayMedix service, the providers simply need to A) confirm eligibility as they normally do, B) provide care for all PayMedix members without needing to arrange for consumer payment, and C) receive full payment. PayMedix handles collecting the balance due from the patient for the portion of the medical charge that is owed by the patient after insurance has paid its part.

PayMedix does not pay incentives or fees to providers.

The agencies also seek information regarding how plans and issuers' billing and reimbursement practices affect health care providers' decisions to offer and promote medical payment products.

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

The agencies seek additional information on the prices or versions of standard charges offered to patients who use these products, and whether these charges are adequately disclosed in accordance with hospital price transparency requirements and No Surprises Act good faith estimate requirements.

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

The PayMedix Guaranteed Universal Billing and Payment Service is offered by employers, not providers. There are no fees or interest charged to consumers. Since the PayMedix service and similar models do not involve the provider's staff in driving enrollment or communicating the benefits of participation to patients, and since there are no consumer fees or interest being charged, there are no disclosures from the provider to the patient.

The agencies seek information on whether medical payment product companies are operating outside of protections against credit reporting of medical collections items and against extraordinary collection actions by tax-exempt hospitals.

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

In the PayMedix Guaranteed Universal Billing and Payment Service model, unpaid balances are considered medical debt, not consumer debt as is the case with many types of medical payment products.

Finally, the agencies seek to better understand how notice and consent requirements for post-stabilization and non-emergency health care items or services under the No Surprises Act intersect with providers' promotion of medical credit cards and loans to out-of-network patients.

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

The PayMedix Guaranteed Universal Billing and Payment Service is offered by employers, not providers. Since the PayMedix service and similar models do not involve the provider's staff in driving enrollment or communicating the benefits of participation to patients, there are no notice or consent requirements for the provider to give patients in the PayMedix model. There are also no fees or interest being charged to consumers in the PayMedix model.

In general, the agencies welcome any information that allows us to better understand the impact of medical payment products on patients' physical, mental, and financial health.

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model. "Medical payment products" is defined in the RFI as those "being administered by financial services companies, who manage the billing and collections process for these products and earn revenue through interest and fees." We do not charge patients any interest or fees. We would suggest adding the word "limited" before "products" in this definition since many medical payment products are limited to certain individuals (e.g., those with higher credit scores), limited in duration (e.g., 12 months), and/or limited by a dollar cap (e.g., $2,000). The PayMedix model has none of those limitations.

Given the above, most medical payment products will not help many consumers due to the limitations inherent in most of them. A Guaranteed Universal Billing and Payment Service like PayMedix, however, will. The starts by including ALL consumers at a given employer regardless their individual credit ratings. The result is clear -- while a medical credit card typically offers its revolving line of credit to very few subprime consumers (<620 credit score), 25% of PayMedix participants are in the subprime category. This includes 10% with a credit score less than 480. Most of these individuals would not access needed care without a Guaranteed Universal Billing and Payment Service like PayMedix.

Providing access to care for lower-income individuals obviously improves their physical, mental, and financial health. The ultimate data point regarding this is that PayMedix employer clients have seen annual trend increases of 2.5-3.4% -- about half what the market has experienced overall. When people can get all of the care when they need it, health care issues are handled earlier. This improves consumers' health but also saves them and their employers money.

Operationally, the PayMedix Guaranteed Universal Billing and Payment Service brings multiple benefits to the patient, including a monthly billing statement that displays all medical charges

for the participant and family members. PayMedix's service includes zero interest or fees due from the patient and offers payment plans customized to the unique circumstances of the family, which helps patients have lower cost of care. Employees have no incentive or encouragement to use more expensive providers or consume unnecessary care outside of their in-network providers. PayMedix is offered to all employees equally, provides an opportunity to dispute and correct erroneous charges, and supports patients in applying for financial assistance at no cost to the consumer. The PayMedix service helps all consumers afford the care they need, thus helping improve their physical and mental health.

The agencies also welcome suggestions of actions Federal agencies could take to address harms caused by medical payment products and related issues connected to medical billing and collections or medical debt more generally.

The Agencies should be lauded for their examination of the expanded use of medical payment products that impose an even greater financial burden on many patients, limit their creditworthiness, and raise overall healthcare costs. CFPB in particular should ensure full transparency for all medical credit cards, installment loans, and other medical payment products that carry interest charges and consumer fees and can cause significant increases in cost of care. Further, the Agencies should ensure that consumers have clear information regarding charity care eligibility and preserve their ability to negotiate bills and secure adjustments.

We strongly urge the Agencies to consider the positive benefits of alternative offerings to traditional medical payment products. One such model that resolves many of the legitimate regulatory concerns of the medical credit card and installment loan category is the Guaranteed Universal Billing and Payment Service offered as an employer solution.

Suggested actions:

1. *Encourage U.S. employers to play a bigger role in helping employees get access to the care they need, including improved ways to pay for the patient's share of medical charges*
    a. *Provide tax advantages for offering Guaranteed Universal Billing and Payment Services to employees*
2. *Clarify the definition of medical payment products to include the word "limited"*
    a. *Medical payment products is defined in the RFI as those "being administered by financial services companies, who manage the billing and collections process for these products and earn revenue through interest and fees". We would suggest adding the word "limited" before "products" in this definition since many medical payment products are limited to certain individuals (e.g., those with higher credit scores), limited in duration (e.g., 12 months), and/or limited by a dollar cap (e.g., $2000)*
        i. *Any limitations like those above should be fully and prominently disclosed to consumers, providers, and employers*

b. *Consider further differentiating between financial products designed for limited or focused use for out-of-network or elective services and solutions designed to address consumer amounts due for in-network allowed charges after insurance*
    i. *For example, this could be as simple as not allowing any product with limitations such as those listed in 1.a above to be used for in-network allowed charges that go through insurance first*
    ii. *Similarly, products that charge consumers interest or fees for in-network allowed charges could be disallowed*

3. *Focus federal rulemaking on simplifying information provided to consumers.*
    a. *It would be easy but counter-productive to significantly expand claim-by-claim disclosures to consumers. In healthcare, consumers are already overwhelmed and end up tuning it all out*
    b. *Instead, focus on consumers not transactions.*
        i. *For example, allow consumers to set their preferences for communications*
            1. *For investment accounts (e.g., at Schwab or Fidelity), consumers can elect to consolidate mandated reporting, assign access and reporting to a spouse, elect electronic not paper delivery, etc.*
            2. *Once preferences are set by the consumer, their agent can execute their instructions on their behalf*
        ii. *Review the intersection of state and federal disclosure requirements.*
            1. *Conflicting or overlapping requirements force market participants to conservatively include absolutely anything that could be required somewhere. This drives more cost as well as consumer confusion*
                a. *It is far better to make sure consumers regularly know the top 5 things that will likely impact them than repeatedly try to tell them the 50 things that might possibly matter very occasionally. Consumers may well take action based on the former, but will tune out and miss the key takeaways from the latter*

4. *Make it clear that any debt that is the result of in-network, post-insurance medical costs should still be considered medical debt regardless of what product financed it. All the rules regarding medical debt should still apply in this case*

a) General Questions

Market Level Inquiries

1. What are the benefits, costs, and risks of medical payment products for consumers, health care providers, and companies offering these products?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model. "Medical payment products" is defined in the RFI as those "being administered by financial services companies, who manage the billing and collections process for these

products and earn revenue through interest and fees." We do not charge patients any interest or fees. We would suggest adding the word "limited" before "products" in this definition since many medical payment products are limited to certain individuals (e.g., those with higher credit scores), limited in duration (e.g., 12 months), and/or limited by a dollar cap (e.g., $2,000). The PayMedix model has none of those limitations.

What PayMedix offers instead is a Guaranteed Universal Billing and Payment Service. This alternative model to medical payment products does not place additional financial burden on patients, preserves their creditworthiness, and treats all patients equitably and with dignity. This billing simplification service is being utilized by a growing number of patients. This model is addressing the payment challenges faced by patients covered by health plans, since 58% of provider bad debt is from patients with health insurance, a 5x increase in 3 years.[19]

In this model, employers contract with a third party to reimburse healthcare providers for the out-of-pocket portion of the bill owed by their employees after they receive medical care. The third-party, for example PayMedix, is then responsible for collecting the out-of-pocket amount owed from the patient, who receives the PayMedix SuperEOB combining all billing and EOB information from all in-network providers they visit — no matter how many or how often — in one consolidated monthly statement. Patients see immediately what they owe, how to pay it, and how to get credit if they need it in the form of extended payment options. The provider is not involved in collecting balances due from these patients.

Employers offer the PayMedix Guaranteed Universal Billing and Payment Service to their employees to enhance their commercial group health insurance, and in particular provide a benefit to employees who have moved to a High Deductible Health Plans (HDHPs). Patients with a HDHP incur higher out of pocket medical expenses until they meet their annual deductible. Unlike medical payment products that are offered to patients after they receive care, patients enrolled in PayMedix know before they receive care that they have the flexibility and benefits of the PayMedix model. This certainty removes a stubborn and persistent source of a barrier care and allows the patient to proceed with needed medical services.

For the employer, the PayMedix model provides an employee benefit that simplifies paying for in-network, out-of-pocket medical expenses for its workforce and reduces the stress and confusion that can further contribute to employee physical health issues. The characteristics and demographics of participating employees in the PayMedix billing service are representative of the full cross section of the employer's workforce. The billing solution is the same for the CEO of the company as it is for an entry level hire.

The PayMedix service brings multiple benefits to the patient, including a monthly billing statement that displays all medical charges for the participant and family members. PayMedix's

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[19] "Hospital collection rates for self-pay patient accounts" (Aug. 2022), *available at* https://www.crowe.com/-/media/crowe/llp/widen-media-files-folder/h/hospital-collection-rates-for-self-pay-patient-accounts-report-chc2305-001a.pdf.

service includes zero interest or fees due from the patient and offers payment plans customized to the unique circumstances of the family, which helps patients have lower cost of care. Employees have no incentive or encouragement to use more expensive providers or consume unnecessary care outside of their in-network providers. PayMedix is offered to all employees equally, provides an opportunity to dispute and correct erroneous charges, and supports patients in applying for financial assistance. The PayMedix service helps consumers afford the care they need, thus helping improve their physical and mental health.

From the perspective of providers, the PayMedix model provides prompt pay for all in network patient balances for each participant opted into the billing simplification service through their employer's benefit program. The provider can focus on clinical and other operational responsibilities of running their practice. They do not promote the PayMedix model to patients, nor does it interfere with charity care or financial assistance programs, and therefore there is less chance that providers will offer unaffordable care that patients neither need nor want.

2. What are the terms of medical payment products, including interest rates and fees?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model. "Medical payment products" is defined in the RFI as those "being administered by financial services companies, who manage the billing and collections process for these products and earn revenue through interest and fees." We would suggest adding the word "limited" before "products" in this definition since many medical payment products are limited to certain individuals (e.g., those with higher credit scores), limited in duration (e.g., 12 months), and/or limited by a dollar cap (e.g., $2,000). Limiting terms like these increase barriers to care for those less well-off, worsening their health and driving up healthcare costs system-wide.

Unlike medical credit cards and installment loans and other medical payment products, the PayMedix Guaranteed Universal Billing and Payment Service does not charge any interest or fees to patients. There are no hidden fees, no teaser rates, and no deferred accrued interest. The cost of the PayMedix service is covered by employers and healthcare providers. Patients pay nothing extra, and removes barriers to accessing early-stage care and produces significant savings over time. If a patient needs a payment plan, PayMedix develops a payment plan that works for the individual's particular needs and covers all of the costs. This never includes interest charges, late fees, origination fees or any other additional expenses for the patient. There are no dollar caps or limits to the duration of a payment plan.

3. How much debt do consumers carry on medical credit cards and loans in total, and what is the average individual debt level?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

4. How concentrated is the medical payment product market, and what role do private equity firms play in this market?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

5. Are there specific populations (e.g., race, socioeconomic status, gender identity, sexual orientation, age, language, etc.) or geographic regions that experience disproportionately higher utilization of medical payment products?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

PayMedix does offer a Guaranteed Universal Billing and Payment Service. Unlike medical credit cards and installment loans targeted for post-service medical expenses, PayMedix does not require each patient to apply or be approved to qualify for the service. There is no review of employee credit history during enrollment and no looking at other data sources that would be used to risk score patients and then adjust pricing and other terms at the individual level. Thus, PayMedix does not "cherry pick" low-risk patients or seduce high-risk patients with gimmicky teaser rates. When an employer selects PayMedix as a benefit, all of its employees are offered credit up to their annual in-network out of pocket maximum with 0% interest and no fees to the patient.

While a medical credit card typically offers its revolving line of credit to very few subprime consumers (<620 credit score), 25% of PayMedix participants are in the subprime category. This includes 10% with a credit score less than 480.

Medical payment products often find patients at a moment of great weakness – when told they need surgery, for example - without the ability to evaluate options or make a thoughtful financial decision. With the PayMedix service already in place, participants need never worry about provider-facilitated financing being offered to them when they can least afford to make a bad choice. This is good for the patient, good for the healthcare provider, and good for society at large.

6. What are the health equity impacts of medical payment products and related billing and collection policies and practices?

See response to Question 5 in General Questions, Market Level Inquiries for the health equity impact of the PayMedix Guaranteed Universal Billing and Payment Service.

i. Do medical payment products affect members of specific underserved communities differently, including members of Tribal communities and geographically isolated communities?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

The PayMedix service includes every member of an employer or group plan equally. No income, demographic, geographic or personal differences among members matter, and all receive the same service and terms.

ii. Do certain products or policies present opportunities to better serve members of underserved communities?

No response provided.

7. Patients can pay for care in many different ways, such as by medical credit card or loan, general purpose credit card, insurance, or through a zero-interest payment plan. What are the costs and benefits for health care providers of offering each of these methods? Are there situations where one method of payment is more advantageous than another?

Historically, consumer healthcare costs in the commercial market (i.e., those not in a government health care program) have been dealt with in two ways:

- ***In-network allowed charges**. These are the co-pays, deductibles, and out-of-pocket amounts for which consumers are responsible under their employer-provided health coverage and which received the best-negotiated price with providers*
    - *A combination of the provider, insurer, and employer could offer financial assistance to consumers in order to pay their portion (e.g., payment plans, HRAs/FSAs/HSAs, health/wellness incentive accounts, etc.)*
    - *Assistance was both **guaranteed and universal** for all consumers regardless their individual financial status*
    - *Consumers had a financial incentive to seek out cost-effective care*

- ***Out-of-network and elective care.** This is care that consumers choose to consume even though it will cost them more money and/or is not a covered medical benefit (e.g., cosmetic surgery)*
    - *If financed, the consumer could individually seek a credit product, or the provider could partner with a financial services firm to offer that function*
    - *Consumers are **individually underwritten**, and whether credit is offered and if so on what terms varies significantly*
    - *Consumer interest and fees are typically a key component of these products*

In the RFI, the Agencies defined medical payment products as those "being administered by financial services companies, who manage the billing and collections process for these products and earn revenue through interest and fees." These were the types of products that traditionally were sold for use in the out-of-network and elective care, and they filled a role in that arena. More recently, though, these medical payment products have been brought to finance the in-network costs after insurance.

Many, including PayMedix, view this expansion of medical payment products as problematic. ***Any product for in-network allowed charges that does not apply to all individuals (regardless of their creditworthiness) and provide full coverage up to a patient's out-of-pocket maximum is cherry-picking consumers with higher credit ratings. This approach diminishes universal access to care, increases health inequity, and drives up overall healthcare costs.*** *A better approach for assisting consumers with their in-network allowed charges is a "Guaranteed Universal Billing and Payment Service". We define and explain this term below.*

This Guaranteed Universal Billing and Payment Service is being utilized by a growing number of patients and is addressing the payment challenges faced by patients covered by health plans – since 58% of provider bad debt is from patients with health insurance, a 5x increase in 3 years.[20] *In this model, employers contract with a third party to reimburse healthcare providers for the out-of-pocket portion of the in-network bill owed by their employees after they receive medical care. The third-party, for example PayMedix, is then responsible for collecting the out-of-pocket amount owed from the patient, who receives the PayMedix SuperEOB combining all billing and EOB information from healthcare providers they visit in one easy-to-read, consolidated monthly statement*[21]*. Patients see immediately what they owe, how to pay it, and how to get credit if they need it in the form of extended payment options. The provider is not involved in collecting balances due from these patients. This alternative model does not place additional financial burden on patients, preserves their creditworthiness, and treats all patients equitably and with dignity.*

A breakdown of the components is as follows:

Guaranteed*. Full payment is guaranteed to the providers on behalf of all consumers*

Universal. All employees and their dependents are automatically included, and all in-network costs up to the full out-of-pocket maximum can be included. No one is excluded for any reason.

Billing*. All of the consumer's provider bills and EOBs are reconciled automatically and are replaced by a simple monthly summary statement*

Payment. *Consumers can pay one bill that covers all of the participating providers, and if they need to pay over time then they can automatically get a no-fee, 0% interest payment plan*

Service. *Since PayMedix is funded by employers and providers, it is provided at no cost to the consumers*

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[20] "Hospital collection rates for self-pay patient accounts" (Aug. 2022), *available at* https://www.crowe.com/-/media/crowe/llp/widen-media-files-folder/h/hospital-collection-rates-for-self-pay-patient-accounts-report-chc2305-001a.pdf.

[21] Employees are able to use the Guaranteed Universal Billing and Payment Service for medical claims generated with healthcare providers who are in-network for their health plan, and also in the PayMedix network.

The Guaranteed Universal Billing and Payment Service approach also helps drive overall system efficiency. Administrative expenses in the U.S. healthcare system take many forms, but one of the most prominent is the process of validating and tabulating amounts owed from providing patient care, and then sending bills and collecting amounts due from patients. Hospitals, physician offices, surgery centers, labs, imaging centers, and other clinical facilities have invested in people and technology tools to accomplish these tasks.

While the adoption of new revenue cycle technology and service solutions by clinical facilities has eased some of the burden of processing medical insurance claims, collecting balances owed by consumers remains a universal and expensive challenge for healthcare providers. Billing teams have invested in portal technologies to enable patient self-service, added payment innovations such as "card on file" to securely store credit card information for future payments, and started to utilize text messaging and robo-dialing to reach patients with overdue balances. Medical billing staff spend an enormous amount of time and money chasing patients with outstanding balances, and risk negatively impacting their patient relationships with the use of intrusive communication tools.

The PayMedix Guaranteed Universal Billing and Payment Service reduces the administrative strain and costs of patient collections for an individual healthcare provider and avoids damaging the provider-patient relationship. In exchange for a modest fee to participate in the PayMedix simplified billing solution, providers avoid incurring the costs associated with collecting overdue balance from patients participating in the PayMedix model. Providers send fewer billing statements, make fewer phone calls, and are able to use their resources to focus on clinical care and ensure the best possible patient outcomes. Providers also appreciate that the PayMedix model is a non-recourse arrangement, so they have certainty of payment with no risk that non-performing patient accounts will "boomerang" back to their balance sheet.

The PayMedix model helps to reduce the cost of patient collections across all participating providers. PayMedix centralizes patient collection activities in one organization and operates a compassionate, efficient collection process at scale. This lowers the unit cost of overall patient collections below what each provider would achieve doing these activities on their own.

8. What incentives do financial services companies offer health care providers, including revenue-sharing or other financial or non-financial incentives?

PayMedix does not offer a medical payment product, thus the following are based on the company's overall market observations.

By contrast, a Guaranteed Universal Billing and Payment Service and similar models do not involve the provider's staff in driving enrollment or communicating the benefits of participation to patients. The PayMedix service is offered by employers to all employees, with no incentives, inducements or other features to motivate providers to push patients into unnecessary care or extra financing costs.

9. How do medical payment products and health insurance coverage interact? Do group health plan or health insurance issuer practices contribute to uptake of medical payment products by patients and providers?

Employers offer the PayMedix Guaranteed Universal Billing and Payment Service to their employees to enhance their commercial group health insurance, and in particular provide a benefit to employees who have moved to a High Deductible Health Plans (HDHPs). Patients with a HDHP incur higher out of pocket medical expenses until they meet their annual deductible. Unlike medical payment products that are offered to patients after they receive care, patients enrolled in PayMedix know before they receive care that they have the flexibility and benefits of the PayMedix model. This certainty removes a stubborn and persistent source of a barrier care and allows the patient to proceed with needed medical services.

The characteristics and demographics of participating employees in the PayMedix service are representative of the full cross section of the employer's workforce. The PayMedix service is the same for the CEO of the firm as it is for an entry level hire. This includes core service features such as the configuration of the consolidated bill (SuperEOB), and the flexibility of the employee to choose the most appropriate bill repayment plan based on their individual financial situation.

i. How many days do providers typically have to wait to be paid by plans or issuers versus by medical payment product companies or general purpose credit card companies? What factors, such as administrative requirements or clinical reviews, contribute to any differential resolution timelines?

PayMedix does not offer a medical payment product, thus the following are based on the company's overall market observations.

Many providers have told PayMedix representatives that various solutions under the RFI definition of "medical payment products" contain surprise fees for them. These are often characterized as "interchange fees" or "electronic payment fees", and providers can get charged them in order to get paid they money they are already owed a little bit faster.

The PayMedix Guaranteed Universal Billing and Payment Service does not charge providers any interchange or electronic payment fees or any surprise fees to participating PayMedix providers. PayMedix has one fully disclosed fee that covers everything related to billing, payment and bad debt risk for all consumers at every employer client.

ii. Does a patient's use of a medical payment product exempt them from certain consumer protections, provider requirements, or group health plan or health insurance issuer requirements? Are different types of health coverage treated differently?

While the three major U.S. credit reporting companies recently agreed to remove medical debt of less than $500 from credit reports, this only begins to address the way these balances affect patient credit. According to the U.S. Census, in 2017, 19% of U.S. households carried medical

debt and the median amount owed was $2,000.[22] One source estimates that most Americans with medical debt owe over $1,000.[23] A substantial number of Americans will still see medical debt negatively affect both their credit scores and their ability to access financing for any needs. It is necessary to find solutions to address the high cost of health care without pushing more consumers to take on onerous and high-interest debt.

Unpaid balances are considered medical debt in the PayMedix model, not consumer debt as is the case with many types of medical payment products.

10. Does health care provider organizational structure, including ownership by private equity, affect providers' decisions to offer and promote these products?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

Provider organizational structure does not matter for the PayMedix Guaranteed Universal Billing and Payment Service. PayMedix and similar models do not involve the provider's staff in driving enrollment or communicating the benefits of participation to patients. PayMedix is offered by employers to their employees, with no incentives, inducements, or other features to motivate providers to push patients into unnecessary care or extra financing costs.

11. What are some best practices for health care providers who offer medical payment products in avoiding adverse financial and health impacts for patients?

As discussed in the answer to Question 7, best practices for providers include:

- *Do not offer or promote medical payment products for the consumer portion of in-network allowed charges that are covered by insurance*
    - *Solutions like a Guaranteed Universal Billing and Payment Service offer a superior alternative for consumers, providers, and employers*
- *Do not require payment before care is rendered for services that will be largely covered by insurance*
    - *Estimates of consumer balances due are difficult to determine due to changed coding details as well as timing of the submission of that particular claim versus others*
    - *Demanding a form of payment up front deters those who are concerned about how to pay for that care*

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[22] U.S. Census, "19% of U.S. Households Could Not Afford to Pay for Medical Care Right Away" (Apr. 2021), *available at* https://www.census.gov/library/stories/2021/04/who-had-medical-debt-in-united-states.html.
[23] KFF-Peterson Health System Tracker, "The state of the U.S. health system in 2022 and the outlook for 2023" (Dec. 2022), *available at* https://www.healthsystemtracker.org/brief/the-state-of-the-u-s-health-system-in-2022-and-the-outlook-for-2023/.

i. Are there specific tactics or practices that are well tailored and adapted for use by health care professionals in and serving underserved communities, including Tribal communities and geographically isolated communities?

No response provided.

ii. What actions should the agencies take to develop and encourage uptake of these established best practices?

No response provided.

iii. Are there examples of actions or best practices at the State or local level to which the Federal government should look?

No response provided.

12. To what extent are patients using medical payment products to pay bills that are incorrect, or that could be covered or defrayed by lower-cost alternatives?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

With the PayMedix Guaranteed Universal Billing and Payment Service, patients can always challenge a bill. The PayMedix customer service team will support the patient in understanding and ultimately disputing any bill with which they disagree. Most customer disputes center on whether the claims should have been paid in the first place. Determining the correct answer to this question is the responsibility of the health plan. PayMedix will work with the health plan and the consumer if the consumer believes a claim should not have been paid and their out-of-pocket amount should have been $0. If the consumer is correct, then PayMedix will process any adjustments and refund the appropriate amount of money to the consumer.

Because providers play no role in introducing or incenting patients to participate in the PayMedix service, they have no conflicted interests when identifying and helping their patients access other local, regional, or federal financial assistance programs. Furthermore, PayMedix helps consumers gather the necessary information and apply for financial assistance from healthcare providers. As of July 2023, PayMedix has assisted patients in qualifying for $750,000 of charity care for over 5,000 individual medical claims. This advocacy offered by PayMedix is communicated on the firm's SuperEOB and elsewhere, and in these cases -- and with the consumer's permission and at no cost to the consumer -- PayMedix can assemble documents and aggregate key qualifying information to the health system on behalf of the consumer. PayMedix is not aware of examples in which medical credit cards and installment loan vendors intervene for their customers to identify charity care relief.

i. What billing errors may patients commonly encounter?

Most customer disputes center on whether the claims should have been paid in the first place. Determining the correct answer to this question is the responsibility of the health plan. PayMedix will work with the health plan and the consumer if the consumer believes a claim should not have been paid and their out-of-pocket amount should have been $0. If the consumer is correct, then PayMedix will process any adjustments and refund the appropriate amount of money to the consumer.

ii. How does using a medical payment product affect patients' rights to dispute incorrect bills?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

With the PayMedix Guaranteed Universal Billing and Payment Service, patients can always challenge a bill. The PayMedix customer service team will support the patient in understanding and ultimately disputing any bill with which they disagree.

iii. Are certain groups of patients, such as members of specific underserved communities, more likely to experience medical billing errors or issues resolving disputes over bills paid using medical payment products?

No response provided.

13. What actions should agencies consider taking to better understand the effects of medical payment products on consumers and the health care industry, educate consumers and providers about the risks of these products, and collect complaints?

No response provided.

i. What are some sources of data on medical payment products? What additional data are needed to understand the impact of medical payment products on patients and the health care industry?

No response provided.

ii. What data collection, data analysis, and research actions should agencies take?

No response provided.

iii. Are there different or other actions that agencies should consider for underserved communities, including Tribal communities and geographically isolated communities?

No response provided.

iv. What types of consumer complaints have States and localities received?

No response provided.

14. Where medical payment products are causing harm, what are some specific levers for regulatory oversight and enforcement by Federal agencies that regulate financial products or health care providers?

No response provided.

i. Are there specific areas for Federal enforcement actions?

No response provided.

ii. Are there examples of regulation or enforcement at the State or local level to which the Federal government should look?

No response provided.

iii. What complementary legislative actions are worth exploring? Where may additional statutory authority be needed?

No response provided.

Individual Inquiries

1. Have medical payment products ever been marketed to you, including by your health care provider? If so, please describe your experience and how the products were marketed to you. Were other options, such as financial assistance, marketed or explained at the same time?

No response provided.

2. If you have used a medical credit card or loan to pay for your care, what was your experience with the product?

No response provided.

a. What benefits or harms did you experience?

No response provided.

b. Was your health affected by your use of a medical credit card or loan?

No response provided.

c. How much did interest and fee charges add to the cost of your care?

No response provided.

d. How did using a medical credit card or loan affect your credit score and your ability to access credit?

No response provided.

e. Would you use a medical credit card or loan to cover medical expenses again? Why or why not?

No response provided.

3. Have you ever tried to dispute a medical bill you paid using a medical credit card or loan? If so, please describe your experience.
No response provided.

4. Have you ever had an overdue bill on a medical credit card or loan sent to collections? How quickly was the bill sent to collections? Did your experience with collections affect your credit score, your access to medical care, or your health?

No response provided.

5. Have you ever felt pressured to pay for care using a medical payment product or general purpose credit card when you believed that was not in your best interest? If so, please describe your experience.

No response provided.

6. Have you ever used or been pressured to use a medical credit card or loan to pay a bill that you believe should have been covered by your health insurance? If so, please describe your experience.

No response provided.

7. Have you ever used or been pressured to use a medical credit card or loan to pay a bill that you believe should have been covered by your health care provider's financial assistance policy? If so, please describe your experience.

No response provided.

8. Has your knowledge about the availability of medical credit cards or loans led you to believe that health insurance might not be necessary, or not acquire health insurance?

No response provided.

b) CFPB-Specific Questions

1. What actions should the CFPB consider taking to address problematic practices related to medical credit cards or loans, including debt collection and credit reporting practices?

The Agencies should be lauded for their examination of the expanded use of medical payment products that impose an even greater financial burden on many patients, limit their creditworthiness, and raise overall healthcare costs. CFPB in particular should ensure full transparency for all medical credit cards, installment loans, and other medical payment products that carry interest charges and consumer fees and can cause significant increases in cost of care. Further, the Agencies should ensure that consumers have clear information regarding charity care eligibility and preserve their ability to negotiate bills and secure adjustments.

We strongly urge the Agencies to consider the positive benefits of alternative offerings to traditional medical payment products. One such model that resolves many of the legitimate regulatory concerns of the medical credit card and installment loan category is the Guaranteed Universal Billing and Payment Service offered as an employer solution.

Suggested actions:

1. *Encourage U.S. employers to play a bigger role in helping employees get access to the care they need, including improved ways to pay for the patient's share of medical charges*
    a. *Provide tax advantages for offering Guaranteed Universal Billing and Payment Services to employees*
2. *Clarify the definition of medical payment products to include the word "limited"*
    a. *Medical payment products is defined in the RFI as those "being administered by financial services companies, who manage the billing and collections process for these products and earn revenue through interest and fees". We would suggest adding the word "limited" before "products" in this definition since many medical payment products are limited to certain individuals (e.g., those with higher credit scores), limited in duration (e.g., 12 months), and/or limited by a dollar cap (e.g., $2000)*
        i. *Any limitations like those above should be fully and prominently disclosed to consumers, providers, and employers*
    b. *Consider further differentiating between financial products designed for limited or focused use for out-of-network or elective services and solutions designed to address consumer amounts due for in-network allowed charges after insurance*

        i. *For example, this could be as simple as not allowing any product with limitations such as those listed in 1.a above to be used for in-network allowed charges that go through insurance first*
        ii. *Similarly, products that charge consumers interest or fees for in-network allowed charges could be disallowed*
3. *Focus federal rulemaking on simplifying information provided to consumers.*
    a. *It would be easy but counter-productive to significantly expand claim-by-claim disclosures to consumers. In healthcare, consumers are already overwhelmed and end up tuning it all out*
    b. *Instead, focus on consumers not transactions.*
        i. *For example, allow consumers to set their preferences for communications*
            1. *For investment accounts (e.g., at Schwab or Fidelity), consumers can elect to consolidate mandated reporting, assign access and reporting to a spouse, elect electronic not paper delivery, etc.*
            2. *Once preferences are set by the consumer, their agent can execute their instructions on their behalf*
        ii. *Review the intersection of state and federal disclosure requirements.*
            1. *Conflicting or overlapping requirements force market participants to conservatively include absolutely anything that could be required somewhere. This drives more cost as well as consumer confusion*
                a. *It is far better to make sure consumers regularly know the top 5 things that will likely impact them than repeatedly try to tell them the 50 things that might possibly matter very occasionally. Consumers may well take action based on the former, but will tune out and miss the key takeaways from the latter*
4. *Make it clear that any debt that is the result of in-network, post-insurance medical costs should still be considered medical debt regardless of what product financed it. All the rules regarding medical debt should still apply in this case*

2. How do firms offering medical financial products typically market to providers?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

The PayMedix Guaranteed Universal Billing and Payment Solution and similar models do not involve the provider's staff in driving enrollment or communicating the benefits of participation to patients. PayMedix is offered by employers to their employees, with no incentives, inducements, or other features to motivate providers to push patients into unnecessary care or extra financing costs.

3. How do creditors and their affiliates underwrite loans to patients? What specific factors (e.g., age, type of medical procedure, credit score, etc.) are considered in underwriting?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

An important feature of the PayMedix Guaranteed Universal Billing and Payment Service is that unlike medical credit cards and installment loans targeted for post-service medical expenses, PayMedix does not require each patient to apply or be approved to qualify for the service. There is no review of employee credit history during enrollment and no looking at other data sources that would be used to risk score patients and then adjust pricing and other terms at the individual level. Thus, PayMedix does not 'cherry pick' low-risk patients or seduce high-risk patients with gimmicky teaser rates. When an employer selects PayMedix as a benefit, all of its employees are offered credit up to their annual out of pocket maximum for all in-network allowed charges with 0% interest and no fees to the patient.

While a medical credit card typically offers its revolving line of credit to very few subprime consumers (<620 credit score), 25% of PayMedix participants are in the subprime category. This includes 10% with a credit score less than 480.

4. Do consumers understand the risks of paying medical bills via a medical credit card, installment loan, or other commercial payment product, including lowered ability to negotiate their bill with their provider?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

The PayMedix Guaranteed Universal Billing and Payment Service makes the patient's out of pocket payment to healthcare providers on behalf of the patient. The patient then pays PayMedix for the out-of-pocket amount that has been paid on their behalf. The patient can reimburse PayMedix in full or can create a payment plan to repay PayMedix over time. Unlike medical credit cards or installment loan products, repayment plans include no interest charges, late fees, hidden costs tied to defined promotional offers or retro-effective interest schemes if payment dates are missed. The patient is simply reimbursing for the face value of the medical bills already paid on their behalf by PayMedix. There is no cost to the patient to participate in the PayMedix service. This is offered as an employer-subsidized benefit to all employees regardless of their individual salary, demographics or credit history.

If a patient needs a payment plan, PayMedix develops a payment plan that works for the individual's particular needs and covers all of the costs. This never includes interest charges, late fees, origination fees or any other additional expenses for the patient. It can extend to the appropriate time period for that particular consumer. Currently, PayMedix members with payment plans average 17 months with a balance of $2311. If they all were in a medical payment product instead – which presumes that they all would have been approved, which is

highly unlikely – then they would need 22 months to pay off the balance at the same monthly payment pace, and have incurred over $575 in interest if at a 25% annual rate.

PayMedix also helps consumers gather the necessary information and apply for financial assistance from healthcare providers. As of July 2023, PayMedix has assisted patients in qualifying for $750,000 of charity care for over 5,000 individual medical claims. This advocacy offered by PayMedix is communicated on the firm's SuperEOB and elsewhere, and in these cases -- and with the consumer's permission -- PayMedix can assemble documents and aggregate key qualifying information to the health system on behalf of the consumer. Medical credit cards and installment loan vendors to our knowledge do not intervene for their customers to identify charity care relief.

With the PayMedix service, patients can always challenge a bill. The PayMedix customer service team will support the patient in understanding and ultimately disputing any bill with which they disagree. Most customer disputes center on whether the claims should have been paid in the first place. Determining the correct answer to this question is the responsibility of the health plan. PayMedix will work with the health plan and the consumer if the consumer believes a claim should not have been paid and their out-of-pocket amount should have been $0. If the consumer is correct, then PayMedix will process any adjustments and refund the appropriate amount of money to the consumer.

5. To what extent are alleged debts placed on medical credit cards and loans sent to debt collectors? How do medical payment product companies' debt collection practices differ from those of health care providers, and are any issuer or provider debt collection practices posing risks to consumers?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

6. How can the CFPB use its authorities to ensure people with medical bills in collections, including medical payment product debt, are screened for eligibility for financial assistance and other benefits?

No response provided.

7. How are health care providers and financial companies using credit or "propensity to pay" scores to determine patients' eligibility for financial assistance or medical payment products? What are the implications for compliance with the Fair Credit Reporting Act or other CFPB authorities?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

An important feature of the PayMedix model is that unlike medical credit cards and installment loans targeted for post-service medical expenses, PayMedix does not require each patient to apply or be approved to qualify for the service. There is no review of employee credit history during enrollment and no looking at other data sources that would be used to risk score patients and then adjust pricing and other terms at the individual level. Thus, PayMedix does not 'cherry pick' low-risk patients or seduce high-risk patients with gimmicky teaser rates. When an employer selects PayMedix as a benefit, all of its employees are offered credit up to their annual out of pocket maximum with 0% interest and no fees to the patient.

While a medical credit card typically offers its revolving line of credit to very few subprime consumers (<620 credit score), 25% of PayMedix participants are in the subprime category. This includes 10% with a credit score less than 480.

PayMedix helps consumers gather the necessary information and apply for financial assistance from healthcare providers at no cost to the consumer. As of July 2023, PayMedix has assisted patients in qualifying for $750,000 of charity care for over 5,000 individual medical claims. This advocacy offered by PayMedix is communicated on the firm's SuperEOB and elsewhere, and in these cases -- and with the consumer's permission -- PayMedix can assemble documents and aggregate key qualifying information to the health system on behalf of the consumer. PayMedix is not aware of examples in which medical credit cards and installment loan vendors intervene for their customers to identify charity care relief.

8. When hospitals write off a patient's debt as uncollectible or "bad debt" and cease attempts to collect, do they notify patients that collection attempts will cease? Would patients benefit from such notifications, and would such notifications reduce hospital revenue?

No response provided.

c) HHS-Specific Questions

1. What actions should HHS consider taking to address problematic practices related to medical credit cards or loans, particularly as they relate to patients eligible for or enrolled in Medicare, Medicaid, or the Children's Health Insurance Program, or patients enrolled in Affordable Care Act Marketplace plans?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model. Also, PayMedix currently only serves the commercial group market, and therefore does not have experience with Medicare, Medicaid, or the Children's Health Insurance Program, or patients enrolled in Affordable Care Act Marketplace plans.

Nevertheless, it is important to note that HHS has recently issued initial guidance regarding the implementation of "smoothing" for Medicare drug plans.[24] *In effect, health plans must offer financing for drug costs to their Medicare members who have drug coverage, and that financing must go all the way to the $2,000 individual cap. All members are to be eligible for this program, and no interest or consumer fees are to be charged.*

Conceptually, this is similar to what the PayMedix Guaranteed Universal Billing and Payment Service has been doing for 15 years. PayMedix has transacted $5 billion through this mechanism, has very high client retention and consumer satisfaction, and has reduced financial barriers to care.

As implementation of the new Medicare drug cost "smoothing" program proceeds, HHS should carefully evaluate which elements of 'medical payment products' would be appropriate and which may not. Health plans and consumers will need clear guidance.

2. What types of health insurance (Medicare, Medicaid, private insurance, etc.) are particularly associated with the likelihood that an individual is offered or makes use of medical credit cards and loans, and how does the type of health coverage affect relevant provider billing practices?

Employers offer the PayMedix Guaranteed Universal Billing and Payment Service to their employees to enhance their benefits program, and in particular provide assistance to employees who have moved to a High Deductible Health Plans (HDHPs). Patients with a HDHP incur higher out of pocket medical expenses until they meet their annual deductible. The PayMedix service is not currently offered to participants in Medicare, Medicaid or non-group commercial plans.

3. Are there particular health care provider types that are most associated with being offered or offering medical payment products, and are these providers receiving directed payments or other incentives through State Medicaid programs?

The PayMedix Guaranteed Universal Billing and Payment Service and similar models do not involve the provider's staff in driving enrollment or communicating the benefits of participation to patients. The PayMedix service is not offered in the Medicaid market.

4. Has the No Surprises Act and its surprise billing protections affected the prevalence and use of medical credit cards and loans, and if so, how?

The No Surprises Act and its surprise billing protections have not affected the PayMedix Guaranteed Universal Billing and Payment Service, since providers are not involved in promoting

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[24] Health Affairs, "Smoothing Medicare Part D Out-Of-Pocket Costs Under The Inflation Reduction Act" (Feb. 2023), *available at* https://www.healthaffairs.org/content/forefront/smoothing-part-d-out-pocket-costs-under-inflation-reduction-act.

the PayMedix service to patients and have no role in disclosures or price transparency rules. PayMedix also charges no fees or interest to consumers.

i. Has the notice and consent process been used to promote medical cards and loans to patients seeking health care items or services from out-of-network providers/facilities, and if so, how? For example, in instances where the No Surprises Act permits providers and facilities to seek notice and obtain consent from an insured patient to waive their balance billing and cost-sharing protections under the No Surprises Act, are providers and facilities impermissibly attaching or incorporating medical card or loan documents or information to the notice and consent forms, or giving them to the patient at the same time as the notice and consent forms?

No response provided.

ii. What steps are health care providers and facilities putting into place to ensure that bills paid through medical payment products do not violate surprise billing requirements and that patients who use medical payment products retain their No Surprises Act rights?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

5. How does or might the use of medical credit cards and loans affect the amount and timing of cost sharing a patient covered through Medicare, Medicaid, and/or the Affordable Care Act Marketplace owes for a covered service?

No response provided.

i. Are there any observable differences in cost sharing among patients belonging to underserved communities, such as Tribal communities or geographically isolated communities?

No response provided.

6. Hospital Price Transparency: What prices or versions of standard charges (e.g., cash prices) are offered to patients who sign up for a medical credit card or installment loan? What steps are taken by health care providers to ensure these charges are adequately disclosed in accordance with hospital price transparency requirements? Do these charges reflect and specifically identify facility fees?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

7. How might HHS improve patient understanding of options for covering the cost of medical treatments? At what points in the care process could patients be provided with information about their financial obligations and payment options?

HHS has made good initial progress with its price transparency efforts.

The question regarding consumers' financial obligations – what percentage of that total cost they may owe – is complicated by timing. Depending upon where a consumer is with respect to deductible and out-of-pocket accumulators, the order in which a series of claims may be submitted by the various providers to the insurance company/TPA and the order in which they are adjudicated can impact the amount of patient obligation remaining for each claim. Putting a "hold" on the consumer balance – similar to how a hotel can put a "hold" on one's credit card when checking in – is impractical when the whole billing and adjudication process can easily take a month.

Consumers today simply do not understand what their EOBs really mean and how they do or do not reconcile with the bills and estimates that they receive from providers. Giving that volume of incomprehensible information to consumers earlier in the process is unlikely to be helpful to them.

As a result, PayMedix has chosen to focus on post-adjudication balances owed by patients. PayMedix replaces the volume of EOBs and provider bills with one simple, monthly statement that summarizes everything that happened. PayMedix members greatly value having One Source of Truth and knowing exactly what they do have as a financial obligation. With that clear understanding, consumers can then focus on determining their payment options.

Anti-Kickback Statute

8. What financial relationships exist between medical payment product companies and health care providers? For example, do companies provide financial incentives to providers who enroll patients in medical payment products? Do providers pay financial companies to collect patients' overdue balances? Or do providers have arrangements with financial companies to indemnify the company in whole or in part if the patient defaults, such as an arrangement that when patients default on their debt to the financial company, the debt reverts to the provider?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

The cost of the PayMedix Guaranteed Universal Billing and Payment Service is covered by employers and healthcare providers. The PayMedix model is a non-recourse arrangement, so providers have no risk that non-performing patient accounts will revert back to their balance sheet.

9. Do health care providers or financial institutions market or recommend medical credit cards or loans to Federal health care program beneficiaries (e.g. Medicare, Medicaid, Affordable Care Act Marketplace, or Children's Health Insurance Program enrollees)? Is the use of these products limited to certain types of health care items or services, such as items and services that are not reimbursable by Medicare or another third-party payor?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

The PayMedix Guaranteed Universal Billing and Payment Solution and similar models do not involve the provider's staff in driving enrollment or communicating the benefits of participation to patients. PayMedix is offered by employers to their employees, with no incentives, inducements or other features to motivate providers to push patients into unnecessary care or extra financing costs. PayMedix does not serve Medicare, Medicaid, ACA Marketplace or CHIP enrollees.

10. Do medical payment product companies recommend certain health care providers to their users? Do companies limit where or how patients use medical credit cards?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

Patients receive the full benefits of the PayMedix Guaranteed Universal Billing and Payment Service when they visit providers who have agreed to participate in the PayMedix provider network.

11. Is the health care provider (or the medical payment product company) offsetting some of the patient's medical debt or providing any other incentives to the patient (e.g., travel rewards for charges to the card)?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

In the PayMedix Guaranteed Universal Billing and Payment Service, employers contract with PayMedix for PayMedix to reimburse healthcare providers for the out-of-pocket portion of the in-network charges owed by their employees after they receive medical care. PayMedix is then responsible for collecting the out-of-pocket amount owed from the patient, who receives the PayMedix SuperEOB combining all billing and EOB information from all in-network providers they visit — no matter how many or how often — in one consolidated monthly statement. Patients see immediately what they owe, how to pay it, and how to get credit if they need it in the form of extended payment options. There are currently no offsets to the patient's medical debt or incentives to the patient from PayMedix.

d) Treasury-Specific Questions

1. What policy actions should Treasury consider taking to address problematic practices related to medical credit cards or loans, including debt collection and credit reporting practices, to conform with the existing tax laws and regulations pertaining to tax-exempt hospitals?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model.

2. Should a tax-exempt hospital's signing patients up for medical payment products be considered similar to a tax-exempt hospital's selling medical debt, such that the special rules that only exclude debt sales from being extraordinary collection actions if certain requirements are met would be applied to these payment products?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model. In general, whether a consumer receivable is, in effect, factored on the front end or the back end of the process should not matter. It would be reasonable to presume that the same special rules would apply. See above to the Introduction section for a description of the PayMedix model.

3. How would applying the debt sale special rules to payment products change hospitals and payment product providers' current practices, especially those related to financial assistance eligibility screening, extraordinary collection actions, interest rates, and recall or return of balances owed by FAP-eligible individuals?

No response provided.

4. How do tax-exempt hospitals' promotion of medical payment products compare to their operationalization of the requirement that their financial assistance policies be widely publicized?

No response provided.

5. What are best practices for hospitals publishing and making patients aware of financial assistance programs (beyond compliance with the widely publicized requirements found in the section 501(r) regulations)?

No response provided.

6. Are medical payment product companies advertising their products as delivering community benefits or as a form of financial assistance?

No response provided.

7. Are tax-exempt hospitals claiming that their promotion of medical payment products delivers community benefits or provides financial assistance, including in their filings of Form 990, Schedule H?

No response provided.

8. Does the availability of medical payment products generally benefit the community or assist patients financially?

PayMedix does not offer a medical payment product, as defined in the RFI, and cannot comment on that model. More generally, however, PayMedix can offer the following perspective and potential actions regarding the broader market:

Historically, consumer healthcare costs in the commercial market (i.e., those not in a government health care program) have been dealt with in two ways:

- ***In-network allowed charges**. These are the co-pays, deductibles, and out-of-pocket amounts for which consumers are responsible under their employer-provided health coverage and which received the best-negotiated price with providers*
    - *A combination of the provider, insurer, and employer could offer financial assistance to consumers in order to pay their portion (e.g., payment plans, HRAs/FSAs/HSAs, health/wellness incentive accounts, etc.)*
    - *Assistance was both **guaranteed and universal** for all consumers regardless their individual financial status*
    - *Consumers had a financial incentive to seek out cost-effective care*

- ***Out-of-network and elective care.** This is care that consumers choose to consume even though it will cost them more money and/or is not a covered medical benefit (e.g., cosmetic surgery)*
    - *If financed, the consumer could individually seek a credit product, or the provider could partner with a financial services firm to offer that function*
    - *Consumers are **individually underwritten**, and whether credit is offered and if so on what terms vary significantly*
    - *Consumer interest and fees are typically a key component of these products*

In the RFI, the Agencies defined medical payment products as those "being administered by financial services companies, who manage the billing and collections process for these products and earn revenue through interest and fees." These were the types of products that traditionally were sold for use in the out-of-network and elective care, and they filled a role in that arena. More recently, though, these medical payment products have been brought to finance the in-network costs after insurance.

Many, including PayMedix, view this expansion of medical payment products as problematic. ***Any product for in-network allowed charges that does not apply to all individuals (regardless of their creditworthiness) and provide full coverage up to a patient's out-of-pocket maximum is cherry-picking consumers. This approach diminishes universal access to care, increases health inequity, and drives up overall healthcare costs.*** *A better approach for assisting consumers with their in-network allowed charges is a "Guaranteed Universal Billing and Payment Service". We define and explain this term below.*

This Guaranteed Universal Billing and Payment Service is being utilized by a growing number of patients and is addressing the payment challenges faced by patients covered by health plans – since 58% of provider bad debt is from patients with health insurance, a 5x increase in 3 years.[25] *In this model, employers contract with a third party to reimburse healthcare providers for the out-of-pocket portion of the in-network bill owed by their employees after they receive medical care. The third-party, for example PayMedix, is then responsible for collecting the out-of-pocket amount owed from the patient, who receives the PayMedix SuperEOB combining all billing and EOB information from healthcare providers they visit in one easy-to-read, consolidated monthly statement*[26]*. Patients see immediately what they owe, how to pay it, and how to get credit if they need it in the form of extended payment options. The provider is not involved in collecting balances due from these patients. This alternative model does not place additional financial burden on patients, preserves their creditworthiness, and treats all patients equitably and with dignity.*

A breakdown of the components is as follows:

Guaranteed. Payment is guaranteed to the providers on behalf of all consumers.

Universal. All employees and their dependents are automatically included, and all in-network costs up to the full out-of-pocket maximum can be included. No one is excluded for any reason.

Billing. All of the consumer's medical bills and EOBs are reconciled automatically and are replaced by a simple monthly summary statement.

Payment. Consumers can pay one bill that covers all of the participating providers, and if they need to pay over time then they can automatically get a no-fee, 0% interest payment plan.

Service. Since PayMedix is funded by employers and providers, it is provided at no cost to the consumer.

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[25] "Hospital collection rates for self-pay patient accounts" (Aug. 2022), *available at* https://www.crowe.com/-/media/crowe/llp/widen-media-files-folder/h/hospital-collection-rates-for-self-pay-patient-accounts-report-chc2305-001a.pdf.

[26] Employees are able to use the Guaranteed Universal Billing and Payment Service for medical claims generated with healthcare providers who are in-network for their health plan, and also in the PayMedix network.

The PayMedix service brings multiple benefits to the patient, including a monthly billing statement that displays all medical charges for the participant and family members. PayMedix's service includes zero interest or fees due from the patient and offers payment plans customized to the unique circumstances of the family, which helps patients have lower cost of care. Employees have no incentive or encouragement to use more expensive providers or consume unnecessary care outside of their in-network providers. PayMedix is offered to all employees equally, provides an opportunity to dispute and correct erroneous charges, and supports patients in applying for financial assistance. The PayMedix service helps consumers afford the care they need, thus helping improve their physical and mental health.

For the consumer portion due from in-network allowed charges after insurance, PayMedix does not believe that medical payment products generally benefit the community or generally assist patients financially. For that, the Guaranteed Universal Billing and Payment Service does deliver community and consumer benefit.

PayMedix does believe that medical payment products have been successfully used for out-of-network care, elective services, and non-medical coverage (e.g., Lasik or dental care). Having them properly continue in that arena could be appropriate.

4. Conclusion

We strongly urge that the CFPB, HHS and Department of Treasury consider the positive benefits of alternative offerings to traditional medical payment products for the consumer portion of in-network allowed charges after insurance. One such model that resolves many of the legitimate regulatory concerns of the medical credit card and installment loan category is the Guaranteed Universal Billing and Payment Service offered as an employer benefit solution. PayMedix is an example of a vendor operating in this emerging category. PayMedix's model is attractive for each of the stakeholders served.

The PayMedix service brings multiple benefits to the patient, including a monthly billing statement that displays all medical charges for the participant and family members. PayMedix's service includes zero interest or fees due from the patient and offers payment plans customized to the unique circumstances of the family, which helps patients have lower cost of care. Employees have no incentive or encouragement to use more expensive providers or consume unnecessary care outside of their in-network providers. PayMedix is offered to all employees equally, provides an opportunity to dispute and correct erroneous charges, and supports patients in applying for financial assistance. The PayMedix service helps all consumers afford the care they need, thus helping improve their physical and mental health.

From the perspective of providers, the PayMedix model provides prompt pay for all in-network patient balances for each participant opted into the billing simplification service through their employer. The provider can focus on clinical and other operational responsibilities of running

their practice. They do not promote the PayMedix model to patients, nor does it interfere with charity care or financial assistance programs.

For the employer, the PayMedix model provides to employees a product that simplifies paying for in-network, out-of-pocket medical expenses for its workforce and reduces the stress and confusion that can further contribute to employee physical health issues. In addition, PayMedix employer clients have seen annual medical trend increases of 2.5-3.4% -- about half what the market has experienced overall. When people can get the care they need it when they need it, health care issues are handled earlier. This not only improves consumers' health but also saves them and their employers money.

Out of pocket healthcare costs present a significant and urgent issue for many Americans. While traditional medical credit cards and installment loans are widely used today, they also present several issues for consumers when used for care traditionally financed by employers, insurers, and providers. It is important that the Agencies recognize alternative, innovative models to lower overall medical billing costs and to ensure that all consumers can afford the care they need.

PayMedix understands that our model is unusual and innovative. We would be happy to meet with the agencies to discuss in more detail how we can work together to help consumers get access to the healthcare they need.

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